

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

February 27, 2026

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

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Chief Medical Officer
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(b)(6),(b)(7)(C),(b)(7)(F) RN
Chief, Investigations Unit
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FROM:

(b)(6),(b)(7)(C),(b)(7)(F) DNP
Senior Investigator, Investigations Unit
ICE Health Service Corps

SUBJECT:

Mortality Review – Report of Findings
Luis Nunez CACERES, (b)(6),(b)(7)(C)

Executive Summary:

On January 5, 2026, the U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of alien Luis Nunez CACERES, (b)(6),(b)(7)(C). ICE detained Mr. CACERES, a 42-year-old Honduran male, at Montgomery Processing Center (MPC), in Conroe, Texas (TX), from November 18 to 25, 2025, and Joe Corley Processing Center (JCPC), in Conroe, TX, from November 25, 2025 until January 5, 2026, the date of his death. Mr. CACERES' autopsy is pending.

Prior to his detention, on an unknown date, Mr. CACERES illegally entered the United States at an unknown place, without being inspected, admitted, or paroled by an immigration officer. On

Mortality Review – Luis Nunez CACERES, (b)(6),(b)(7)(C)

Page 2 of 8

November 17, 2025, ICE Enforcement and Removal Operations (ERO) Houston arrested Mr. CACERES. On November 18, 2025, ERO Houston transferred Mr. CACERES to MPC, in Conroe, TX, and served him with a Notice to Appear, Form I-862, pursuant to sections 212(a)(6)(A)(i) and 212(a)(7)(A)(i)(I) of the Immigration and Nationality Act, as amended.

On November 19, 2025, an MPC registered nurse (RN) completed Mr. CACERES' intake screening, documented his history of hypertension (high blood pressure), and placed him in the medical housing unit (MHU) for respiratory isolation and tuberculosis testing due to an abnormal chest X-ray. On November 21, 2025, an MPC advanced practice provider (APP) completed his comprehensive health assessment, noted his history of congestive heart failure [(CHF); the inability of the heart to pump sufficient blood, leading to inadequate tissue perfusion and fluid congestion], and high cholesterol, ordered medications, and continued his MHU admission. On November 24, 2025, an MPC APP cleared him for general population and transfer after they completed initial testing and started him on tuberculosis medications.

On November 25, 2025, ICE transferred Mr. CACERES to JCPC. On the same date, a JCPC licensed vocational nurse (LVN) completed his medical intake screening and noted his chronic medical conditions, including medications. Between November 29 and December 18, 2025, JCPC medical staff completed his comprehensive health assessment, educated him on taking his CHF medication as prescribed, and treated him for cold symptoms (on three separate occasions).

On December 22, 2025, a JCPC physician referred Mr. CACERES to the Conroe Regional Medical Center (CRMC) emergency room, via custody transport vehicle, for evaluation of his diminished urine output. At approximately 10:30 p.m., shortly after he returned to JCPC, Mr. CACERES became lethargic, diaphoretic, and appeared to experience difficulty breathing. As a result, a JCPC physician ordered his return to CRMC via ambulance.

Between December 23 and 30, 2025, CRMC medical staff diagnosed Mr. CACERES with CHF exacerbation, consulted with cardiology and infectious disease specialists, completed diagnostic imaging, monitored his laboratory values and treated abnormalities as needed. Between December 31, 2025, and January 5, 2026, Mr. CACERES suffered cardiac arrest on four separate occasions. CRMC staff initiated life-saving measures each time and achieved return of spontaneous circulation three times, however, their fourth attempt to resuscitate Mr. CACERES did not succeed. On January 5, 2026, a CRMC physician pronounced Mr. CACERES deceased.

Mortality Finding:

As a result of Mr. CACERES' death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

Mortality Review – Luis Nunez CACERES, (b)(6),(b)(7)(C)
Page 3 of 8

A mortality review committee (MRC) determined MPC and JCPC did not provide Mr. CACERES with medical care within the safe limits of practice. Specifically, the MRC determined: 1) MPC generated a transfer summary with several discrepancies, and JCPC did not document a plan to prevent Mr. CACERES from taking extra doses of furosemide. 2) JCPC delayed Mr. CACERES' comprehensive health assessment. 3) JCPC did not provide appropriate CHF management or order appropriate diagnostic testing after Mr. CACERES self-administered excess furosemide.

The MRC convened on February 13, 2026, and consisted of the following IHSC members: Dr. (b)(6),(b)(7)(C),(b)(7)(F) clinical director, physician reviewer; Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) chief APP, APP reviewer; and CAPT (b)(6),(b)(7)(C),(b)(7)(F) western regional nurse manager, nurse reviewer.

The following summary of events was based on: 1) Mr. CACERES' MPC and JCPC medical records; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. CACERES' MPC and JCPC detention file; 5) staff interviews conducted by Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) virtually on January 29 and February 4, 2026; 6) applicable MPC and JCPC policies and procedures; and 7) applicable ICE Detention standards.

Summary of Events

On November 19, 2025, an MPC RN completed Mr. CACERES' intake screening, documented his history of hypertension treated with carvedilol and losartan, recorded normal vital signs, and completed his nursing admission to the medical housing unit for respiratory isolation and tuberculosis testing due to an abnormal chest X-ray.

On November 21, 2025, an MPC APP completed Mr. CACERES' comprehensive health assessment, documented his history of hypertension, high cholesterol, and CHF, and recorded normal vital signs. The APP completed his provider admission to the medical housing unit with regular nursing checks and vital signs, submitted a cardiology referral, ordered medications to treat tuberculosis, continued his medications to treat hyperlipidemia, hypertension, and CHF, based on community pharmacy records, and ordered baseline laboratory studies and an EKG, which showed a first degree atrioventricular block (abnormal heart rhythm in which electrical signals throughout the heart are delayed) and left atrial enlargement.

Between November 21 and 24, 2025, Mr. CACERES remained in the MPC medical housing unit for tuberculosis testing and medications.

On November 24, 2025, an MPC APP evaluated Mr. CACERES, reviewed his baseline laboratory study results, diagnosed him with diabetes mellitus (DM), and prescribed metformin (treats DM) 500 milligrams (mg) extended-release tablet, with evening meals, daily. The APP

Mortality Review – Luis Nunez CACERES, (b)(6),(b)(7)(C)

Page 4 of 8

documented Mr. CACERES' tolerance of tuberculosis medications, noted his sputum samples did not show positive results for tuberculosis on initial testing, and cleared him for general population and transfer.

On November 25, 2025, ICE transferred Mr. CACERES to JCPC.

- On the same date, a JCPC LVN completed Mr. CACERES' medical intake screening and documented his history of DM, hypertension, high cholesterol, a heart condition (unspecified), and lung surgery thirteen years ago. The LVN noted normal vital signs, except an elevated blood sugar reading of 212 milligrams per deciliter [(mg/dL); normal 60 – 125 mg/dL], Mr. CACERES' prescriptions for tuberculosis treatment, and the following active medications: atorvastatin (treats hyperlipidemia) 10 mg, one tablet, orally, at bedtime; carvedilol (treats hypertension and CHF) 6.25 mg, one tablet, orally, twice a day; apixaban (blood thinner) 5 mg, one tablet, orally, twice a day; furosemide (treats hypertension and CHF) 40 mg, one tablet, orally, daily; losartan (treats hypertension) 25 mg, one tablet, orally, daily; metformin 500 mg, one tablet, orally, at bedtime; spironolactone (treats CHF) 25 mg, one tablet, orally, daily; and vitamin B6 50 mg, one tablet, orally, in the morning.

On November 29, 2025, an RN conducted a routine sick call evaluation for Mr. CACERES' complaints of congestion, chills, and pain. The RN recorded normal vital signs, clear lung sounds, and his denial of chest pain and difficulty breathing, provided acetaminophen (treats pain), instructed him to increase his oral fluids and rest, and notified an APP. The APP provided a verbal order for loratadine (treats seasonal allergies) 10 mg, one tablet, orally, at night.

On December 4, 2025, an APP completed Mr. CACERES' comprehensive health assessment and noted his previously reported medical history, including coronary artery disease with coronary artery bypass grafting in 2018. The APP noted a normal physical exam and vital signs, and ordered an EKG and baseline laboratory tests, including a lipid panel, complete blood count, comprehensive metabolic panel, and hemoglobin A1c.

- On the same date, an LVN conducted a sick call evaluation for Mr. CACERES' complaints of cold symptoms and sore throat for five days, weakness, chills, fatigue, and congestion. The LVN documented clear lung sounds and vital signs within normal limits, administered acetaminophen 325 mg, two tablets, orally, twice a day as needed, for five days, and instructed him to gargle one teaspoon of salt in eight ounces of warm water three to four times a day for three days.

On December 18, 2025, at approximately 3:50 p.m., an APP documented Mr. CACERES presented to the medical clinic for medication education after he prematurely ran out of furosemide. The APP noted he reported taking two tablets daily, instead of the one tablet prescribed daily, complained of cold symptoms, and denied chest pain. The APP recorded

Mortality Review – Luis Nunez CACERES, (b)(6),(b)(7)(C)

Page 5 of 8

normal vital signs, documented a normal physical assessment, except for bilateral leg swelling, ordered furosemide 40 mg, one tablet, orally, daily, loratadine 10 mg, one tablet, orally, at bedtime, and cough syrup, twice a day, and gave him compression stockings.

On December 22, 2025, at 1:50 p.m., a custody officer called a medical emergency for Mr. CACERES' complaints of bilateral lower extremity swelling and painful urination (dysuria). Health staff responded, transported Mr. CACERES to the medical clinic on a stretcher, and documented normal vital signs, except a low pulse of 49 beats per minute [(bpm); normal 60 – 100 bpm], his report of no urine output (anuria) for the past two days, and a normal physical exam.

- At approximately 2:00 p.m., one of the responding health staff members received a physician order to send Mr. CACERES to Conroe Regional Medical Center (CRMC) emergency room, via custody transport vehicle, for dysuria and anuria evaluation.
- At approximately 10:30 p.m., a licensed practical nurse (LPN) completed Mr. CACERES' off-site return evaluation and documented he became lethargic, diaphoretic, and appeared to have difficulty breathing. The LPN administered 5 liters of supplemental oxygen via non-rebreather mask, noted normal vital signs, except rapid respirations of 24 breaths per minute, and pulse of 101 bpm, abnormal EKG, and sent Mr. CACERES back to CRMC, via ambulance, per physician order.

On December 23, 2025, at approximately 4:53 a.m., a JCPC LPN documented CRMC admitted Mr. CACERES for congestive heart failure exacerbation.

Between December 23 and 30, 2025, CRMC staff continuously evaluated Mr. CACERES' condition, consulted with cardiology and infectious disease specialists, completed diagnostic imaging, and monitored his laboratory values and treated abnormalities as needed.

On December 31, 2025, Mr. CACERES suffered a cardiac arrest. CRMC staff identified an abnormal heart rhythm (ventricular fibrillation), initiated life-saving measures, including defibrillation and intubation, achieved return of spontaneous circulation, and transferred him to the intensive care unit. A CRMC physician later noted Mr. CACERES' low potassium level and started potassium replacement therapy. Following Mr. CACERES' cardiac arrest, CRMC staff noted intent to collaborate with ICE to contact next-of-kin and noted his overall "very poor" prognosis.

Between January 1 and 4, 2026, CRMC medical staff stabilized Mr. CACERES' condition, continually monitored his laboratory values, administered medications, and consulted with cardiology, infectious disease, and nephrology specialists.

Mortality Review – Luis Nunez CACERES, [(b)(6),(b)(7)(C)]

Page 6 of 8

On January 5, 2026, between 1:35 and 4:27 a.m., Mr. CACERES experienced sudden cardiac arrest on three occasions. CRMC staff implemented life-saving measures and achieved return of spontaneous circulation twice; however, CRMC staff's third attempt to resuscitate Mr. CACERES did not succeed.

- At 4:31 a.m., a CRMC physician pronounced Mr. CACERES deceased.

The MRC used the following accreditation standards for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 revisions; and National Commission on Correctional Health Care Standards for Health Services in Jails (NCCHC), 2018. The MRC identified the following program weaknesses and recommendations during this review.

MPC and JCPC Weaknesses

1. Documentation

- On November 24, 2025, the medical transfer summary an MPC RN completed contained multiple discrepancies. The RN documented a normal chest X-ray dated November 13, 2025; however, the actual radiology report, completed at MPC and dated November 19, 2025, showed an abnormal finding suspicious for tuberculosis. In addition, the RN documented “No” for tuberculosis despite Mr. CACERES’ active treatment. Although Mr. CACERES had a pending cardiologist appointment for an echocardiogram, the RN noted no pending appointments on the transfer summary.
- On December 18, 2025, Mr. CACERES requested an early refill of his furosemide. He reported taking two tablets per day instead of one tablet as prescribed. During staff interviews, the JCPC APP who evaluated him for the refill stated he reordered the medication and required nursing staff to administer it under a direct observation therapy (DOT) to prevent further self-administration error; however, the APP did not document the DOT requirement in the treatment plan.

Applicable standards of care for this finding:

- NCCHC: Standards for Health Services in Jails, 2026: J-A-08, *Health Records*.
- PBNDS 2011 with 2016 revisions: Part 4.3, Medical Care; section BB. *Medical Records*.

2. Delay of Care

- JCPC electronic health records system contains a task management function that assigns clinical tasks with defined due-date parameters based on patients’ medical history. The

Mortality Review – Luis Nunez CACERES, (b)(6),(b)(7)(C)

Page 7 of 8

task protocol explicitly requires providers to complete history and physical examinations within one day for patients with infectious diseases and chronic medical conditions, such as hypertension, diabetes, and heart disease; however, the provider did not complete the physical examination until 10 days after the task assignment date.

Applicable standards of care for this finding:

- NCCHC: Standards for Health Services in Jails, 2026: J-A-08, *Health Records*.
- PBNDS 2011 with 2016 revisions: Part 4.3, Medical Care; section BB. *Medical Records*.

3. Inappropriate Medical Care

- During staff interviews, the JCPC APP who completed Mr. CACERES' comprehensive health assessment acknowledged clinical suspicion of CHF; however, he did not initiate appropriate diagnostic evaluation (echocardiogram), and daily weight monitoring, which allows providers to detect early fluid accumulation, and enable timely clinical intervention, including adjustment of diuretic therapy.
- On December 18, 2025, Mr. CACERES reported taking extra doses of his prescribed furosemide. The JCPC APP who evaluated him confirmed during interview his awareness of the extra doses; however, the APP did not order diagnostic testing (electrolyte imbalance blood analysis and EKG) to rule out complications associated with furosemide overdose, such as low potassium, which could cause abnormal heart rhythms and sudden cardiac arrest.

Applicable standards of care for this finding:

- Heidenreich, Paul A., Biykem Bozkurt, David Aguilar, Larry A. Allen, Joni J. Byun, Monica M. Colvin, Anita Deswal, et al. 2022. "2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure." *Journal of the American College of Cardiology* 79 (17). <https://doi.org/10.1016/j.jacc.2021.12.012>.
- NCCHC: Standards for Health Services in Jails, 2026: J-B-08, *Patient Safety*.
- NCCHC: Standards for Health Services in Jails, 2026: J-D-02, *Medication Administration Services*.
- NCCHC: Standards for Health Services in Jails, 2026: J-D-07, *Hospital and Specialty Care*.
- NCCHC: Standards for Health Services in Jails, 2026: J-E-09, *Continuity, Coordination, And Quality of Care During Incarceration*.
- NCCHC: Standards for Health Services in Jails, 2026: J-F-01, *Chronic Disease Management and Specialized Health Care Services*.
- NCCHC: Standards for Health Services in Jails, 2026: J-A-01, *Access To Care*.

Mortality Review – Luis Nunez CACERES, (b)(6),(b)(7)(C)
Page 8 of 8

Recommendations for MPC

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Systems Support Division (HSSD), DAD of Clinical Services Division (CSD), and DAD of Health Care Compliance Division (HCCD).
- The IHSC DAD of HSSD will share these findings with MPC's health services administrator (HSA) for review, dissemination to ICE and MPC's facility administrator, and any action(s) deemed appropriate.
- The IHSC DADs of CSD and HCCD will share these findings with the respective IHSC CSD and HCCD units for review and any action(s) deemed appropriate.
- MPC's HSA and the respective IHSC CSD and HCCD units will ensure a corrective action plan, if applicable, is implemented and sustained.

Recommendations for JCPC

- Forward these findings to the IHSC DAD of HCCD.
- The IHSC DAD of HCCD will share these findings through appropriate communication channels to ICE, and JCPC's facility administrator and health authority, for review and any action(s) deemed appropriate.
- The respective IHSC HCCD unit and ICE will ensure a corrective action plan, if applicable, is implemented and sustained.

End of report.